

Suspected Stroke Algorithm



Adult Advanced Cardiovascular Life Support



Identify signs and symptoms of possible stroke
Activate Emergency Response

Critical EMS assessments and actions

- Support ABCs; give **oxygen** if needed
- Perform prehospital stroke assessment
- Establish time of symptom onset (last normal)
- Triage to stroke center
- Alert hospital
- Check glucose if possible

NINDS
TIME
GOALS

ED
Arrival



ED
Arrival



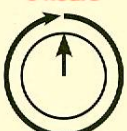
ED
Arrival



ED
Arrival
60 min



Stroke
Admission
3 hours



A
B
C
D
E

Immediate general assessment and stabilization

- Assess ABCs, vital signs
- Provide **oxygen** if hypoxemic
- Obtain IV access and perform laboratory assessments
- Check glucose; treat if indicated
- Perform neurologic screening assessment
- Activate stroke team
- Order emergent CT scan or MRI of brain
- Obtain 12-lead ECG

Immediate neurologic assessment by stroke team or designee

- Review patient history
- Establish time of symptom onset or last known normal
- Perform neurologic examination (NIH Stroke Scale or Canadian Neurological Scale)

Does CT scan show hemorrhage?

No Hemorrhage

Hemorrhage

Probable acute ischemic stroke; consider fibrinolytic therapy

- Check for fibrinolytic exclusions
- Repeat neurologic exam: are deficits rapidly improving to normal?

Consult neurologist or neurosurgeon; consider transfer if not available

Patient remains candidate for fibrinolytic therapy?

Not a Candidate

Candidate

Administer aspirin

Review risks/benefits with patient and family. If acceptable:

- Give **rtPA**
- No anticoagulants or antiplatelet treatment for 24 hours

- Begin stroke or hemorrhage pathway
- Admit to stroke unit or intensive care unit

- Begin post-rtPA stroke pathway
- Aggressively monitor:
 - BP per protocol
 - For neurologic deterioration
- Emergent admission to stroke unit or intensive care unit

